جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : هوايد رمضان عطيه جعفر
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ~م القديمه~
رقم البطاقة : 27304080103442 **رقم التليفون :** 01143331580 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 08-04-1973 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: هوايد رمضان عطيه جعفر
National ID: 27304080103442

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	هويدا رمضان ابراهيم	Age	52
Sex		Special habits	
Family History	-ve	Comorbidity	IHD, HTN
Surgical History		Telephone	01143331580
Menopausal Status	post	Marital Status	widow
ID	27304080103442	Medical ID	2099/2025
Date	10/08/2025	BMI	

Medical Report

MDT

Pathology: Bx-> Left invasive mammary cancer GII

ER: 5, PR: 4, Her2: 0, Ki67: 40%

S.M: Lt UOQ Birads 5, at 2 oclock LN +ve. Bilateral breasts mild fibroadenosis and mild duct ectasia Birads 2. For Dynamic contrast MRI breast study to detect multicentricity

Metsworkup: CTCAP-> Left breat masses, L3 hypodense lesion likely benign nature for bone scan correlation. No evidence of hepatic or pulmonary mets

Recommendations

MDT DECISION	- Bone scan - Clip insertion - NACT AC X 4 - Taxol 12 weeks

Name	د/ حنان عزت	د/ احمد نجيب
Signature		

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: هويدا رمضان عطيه جعفر **MR. :** 6945	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with locally advanced breast cancer in August 2025 and was scheduled for neoadjuvant chemotherapy (NACT) after a bone scan and clip insertion. The patient missed follow-up visits until the present assessment, at which time a CT of the chest with contrast revealed a large left breast mass measuring approximately 5 × 6 cm with no palpable lymph nodes.

On 21 June 2026 the patient was noted to have a 1.98 cm post-mastectomy left breast cancer, T3 N1, with an IDC 2 ER 5/8 PR 4/8 HER2-negative, Ki-67 40 %. Clips were inserted during the left breast examination. The left upper outer quadrant mass measured 3 × 4 cm. A CT of the spine showed a lytic lesion at D10–L3; bone scan pending. The patient was at 64 % response on 21 June for cycle 3 of AC + Adriamycin 120 mg and Endoxan 1200 mg, with neoadjuvant status 1.98.

On 12 July 2026 the patient received cycle 4 of AC + Adriamycin 120 mg and Endoxan 1200 mg over 1 hour with soilucortif as neoadjuvant therapy, with labs and supportive treatment completed.

On 2 August 2026 the patient was examined with a 3 × 2 cm left upper outer quadrant mass and no axillary lymph nodes felt. The patient was started on week 1 taxol 150 mg as neoadjuvant therapy; week 2 taxol 3 ph (presumably 3 phases) with bone scan pending.

On 23 August 2026 the patient received week 4 taxol; labs were accepted with CBC pending. The patient was also started on bional and solucortef for weeks 5 and 6, with pharmacy involvement.

On 30 August 2026 the patient had a peripherally inserted central catheter (PICC) line 2 for taxol. The dose was reduced by 20 % to 120 mg for week 5 taxol, with supportive therapy (methyl-techno) added. Solucortef was added for week 6 taxol, with pharmacy coordination.

On 6 September 2026 the patient, known to have rheumatoid arthritis, received week 5 taxol 110 mg with supportive therapy. The patient complained of eyelid nodules and multiple skull lesions; an MRI of the brain was ordered for weeks 6 and 7 in the pharmacy.

The most recent note indicates that the patient has completed 2 cycles of AC + Adriamycin (C3 and C4) and 6 weeks of taxol (W1, W2, W4, W5, and W6).

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

اسم المريض: هوايدا رمضان عطية جعفر

التاريخ: 2025/05/17

Digitalized Low Dose Soft Tissue Mammography With Complementary Breast US Examination Of Both Breasts Revealed:

- ACR B: Mixed fibro-fatty-glandular breast parenchyma.
- Left breast shows UOQ two dense speculated mass lesions corresponding to ultrasound two large adjacent ill-defined speculated globular heterogeneous hypoechoic lesions about 2 mm in-between with internal vascularity by color Doppler at 2 o'clock position, they measuring about 37.5 x 25 mm and 29 x 17 mm with overall measuring about 55 x 27 mm. they are seen abutting overlying skin and underlying muscles.
- Bilateral mildly dilated retroareolar ducsts.
- No right breast solid or cystic lesions.
- Normal skin thickness and contour as well as nipple areola complexes on the right side.
- Few enlarged left axillary LNs showing increased cortical thickness and indented fatty hilum. The largest measures 29 x 16.5 mm with 8.1 mm cortical thickness.
- Right non-specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Left breast UOQ malignant lesions as described (BIRADS 5); for ultrasound guided biopsy.**
- **Bilateral breasts mild fibroadenosis and mild duct ectasia (BIRADS 2).**
- **For Dynamic contrast MRI breast study to deteet multicentricity/ multifocality.**

Best regards,
Dr. Ayat Nadi, FEBR, ABR

Sonography
[[DOCTYPE:SONOGRAPHY]]

NHTMRI

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجي

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: هويدا رمضان عطيه جعفر
Request type: مبادره
Hospital no: صحة المراه
Department:
Referred by: د.امل
Nature of specimen: Left Breast Mass, Tru-cut biopsy.

Age: 52 years
Sex: Female
Sub request type:
Pathology no.: 1528-2025
Date of Receiving: 20/1/2025
Receiving date: 21/1/2025
I.D. Number: 29304080103442

Clinical diagnosis: Left breast 2 o'clock suspicious mass lesion.

Gross pathology :
Multiple grayish white tissue cores up to 1.7cm long, totally submitted.

Immunohistochemical Technique:
Sections were prepared from the paraffin block, processed in benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were tested against: ER- PR- Her2- Ki67

Microscopic picture:
Core examined revealed infiltrating mammary carcinoma of solid forms sheets, clusters, cords and trabeculae infiltrating among desmoplastic stroma with marked lymphocytic response. Tumor cells show moderate anaplasia with moderate amount of forming tubules and mitosis. with lymphovascular emboli, but no perineural invasion could be seen in the edition cort tissue examined.

Immunohistochemistry:
ER: Positive moderate nuclear staining in 30% of tumor cells (Allred Scores 5/8)
PR: Positive moderate nuclear staining in <10% of tumor cells (Allred Scores 4/8)
Her2: No membranous staining of tumor cells (Score 0)
Ki67: Positive nuclear staining of 40% of tumor cells

Diagnosis:
Left breast mass, Tru cut biopsy:
Invasive Mammary Carcinoma, Grade II

Signatures:
Prof. Dr. Mamdouh Mahdy
Dr. Nermine Nabil, M.D
Dr. Mona Mohamed, MD

Pathology Lab. Manager

Nermine Nabil, M.D

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال

ECHO REPORT

NAME: وفية محمد رمضان

DATE: 16 / 8 /2024

ID NUMBER: /

SEX: AGE:

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.6	2-4cm	LA	3.7	2-4cm
FS	29%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal

SWMA No. RWMA

LA and Ao Root Dimensions Normal

Mitral Valve N

Aortic Valve N

Tricuspid Valve N

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR.

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Medical No. : **4042**
Patient's Name: **Howaida Ramadan Atieh Jaafar**
Date: **2026-09-20**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

Duplex Examination of the venous systems of left lower limb revealed: -

***Deep Venous System*: -**

- *The femoral (common & superficial), popliteal & infrapopliteal veins of the examined lower limb all are Patent, totally compressible with normal color filling & preserved respiratory variation.*
- *No evidence of D.V.T at time of scan.*

***Superficial Venous System*: -**

- *Patent long & short saphenous veins with no evidence of superficial thrombophelebitis at time of scan.*

OPINION: -

No evidence of acute D.V.T or superficial thrombophelebitis at time of scan

Mild interstitial edema of the examined limb

DR: Nermeen Ramadan.MD

[[DocType:CT_Scan]]
CT Scan

Medical No. : 4042
Patient's Name: Howaida Ramadan Atieh Jaafar
Date: 2026-09-17
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

- **MRI of the brain with contrast**

Technical data:

- Routine pulse sequences with contrast

Comparison: none

Findings:

- Posterior high parietal midline scalp well defined lesion of high T1 signal 3x2 cm with no calvarial or deep extension, needs further evaluation by fat sat sequences to assess its fat signal/?lipoma
- Other multiple scalp well defined low T1 and T2 signal smaller lesions not exceeding 2 cm, no calvarial or deep extension (likely benign looking, possible trichilemmal cysts)
- Brain involutional changes in form of mildly dilated ventricular system and prominent extra axial CSF spaces
- Normal appearance of the cerebral hemispheres parenchyma with no focal mass lesion or areas of abnormal signals.
- Preserved gray white matter interface
- No midline shift.
- No extra axial fluid collections.
- Normal appearance of the cerebellar hemispheres and brainstem with no focal lesions or areas of abnormal signal.
- No brain pathologically enhancing lesions.

Conclusion:

- **Posterior high parietal midline scalp well defined lesion of high T1 signal 3x2 cm with no calvarial or deep extension, needs further evaluation by fat sat sequences / plain CT to assess its fat signal/ density ?lipoma**
- **Other multiple scalp well defined low T1 and T2 signal smaller lesions not exceeding 2 cm, no calvarial or deep extension (likely benign looking, possible trichilemmal cysts, fibrous lesions, burned out lesions?)**
- **No brain pathologically enhancing lesions.**
- **Age related brain involutional changes**

Regards, Dr. Ahmed Rezk, MD

Ahmed

[[DOCTYPE:BONE_SCAN]]
Bone Scan

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Howaida Ramadan Atieh Jaafar	**Date : 10-09-2026 08:43**
Branch : Dokki	**Medical No. : 458588**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : تعاقد مستشفيات**

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 53-year-old female with right breast carcinoma, status post chemotherapy (last September 2022). Presents with bony aches. Referred for baseline/follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Homogeneous symmetric physiological tracer distribution throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Negative bone scan. No scintigraphic evidence of skeletal metastases.
- Follow up as per clinical pathway.

Dr. Alaa Nagi

MD Nuclear Medicine

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال مصر

ECHO REPORT

NAME : سعيدا رضا مرضية DATE: 4 /2 /2026

ID NUMBER : / SEX : F AGE: 52

EF	68%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.5	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve N

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR. محمد الخطيب

Echo
[[DOCTYPE:ECHO]]

GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال مصر

ECHO REPORT

NAME: هويدا رضا وطية

DATE: 21 / 6 /2026

ID NUMBER: /

SEX: AGE:

EF	64%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.5	2-4cm
FS	28%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve N

Tricuspid Valve N

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR.

Echo
[[DOCTYPE:ECHO]]

[[DOCTYPE:MDT_FORM]]
MDT Form

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	هويدا رمضان ابراهيم	Age	52
Sex		Special habits	
Family History	-ve	Comorbidity	IHD, HTN
Surgical History		Telephone	01143331580
Menopausal Status	post	Marital Status	widow
ID	27304080103442	Medical ID	2099/2025
Date	10/08/2025	BMI	

Medical Report

MDT

Pathology: Bx-> Left invasive mammary cancer GII

ER: 5, PR: 4, Her2: 0, Ki67: 40%

S.M: Lt UOQ Birads 5, at 2 oclock LN +ve. Bilateral breasts mild fibroadenosis and mild duct ectasia Birads 2. For Dynamic contrast MRI breast study to detect multicentricity

Metsworkup: CTCAP-> Left breat masses, L3 hypodense lesion likely benign nature for bone scan correlation. No evidence of hepatic or pulmonary mets

Recommendations

MDT DECISION	- Bone scan - Clip insertion - NACT AC X 4 - Taxol 12 weeks

Name	د/ حنان عزت	د/ احمد نجيب	
Signature			